

Public Health Emergency Operations Center
"COUSP DRC"

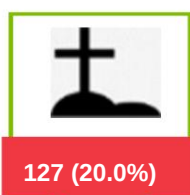
MVE-17 Incident Management System

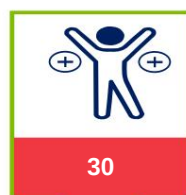
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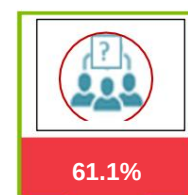
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°26/MVB_09/06/2026

Provinces affected (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (26)	- Ituri (18/36): Aru, Aungba, Bambu, Bunia, Damas, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia. - North Kivu (7/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha - South Kivu (1/34): Miti-Murhesa
Reporting date	June 9, 2026
Publication date	June 10, 2026


 Total confirmed cases
for the 3 provinces

 Cumulative deaths
among confirmed cases
and case fatality rate

 Patients in
isolation -
hospitalization

 Cumulative number
of recoveries

 Suspected cases of
the day

 Contact tracing rates
for the 3 provinces

*Data is being harmonized to include confirmed cases still in the community and cases of evasion in the overall count

Suspicious deaths occurring in the community are subject to a thorough investigation with a view to possible reclassification as probable cases.

1. HIGHLIGHTS

- As of June 9, 2026, **37 new confirmed cases, including 12 deaths**, were reported in the health zones of **Mongbwalu (13), Rwampara (11), Bunia (10), Tchomia (2) and Lita (1)**, in the province of Ituri.
- One (1)** new health zone has been assigned in the province of Ituri: this is the Health Zone of Tchomia.
- Eight (8) confirmed cases** of Ebola virus disease, including 7 in the Nyankunde Health Zone and 1 in Mongbwalu (Ituri province) have been declared cured.
- Launch of medical care activities in two (2) Ebola treatment centers (HGR) Bunia and HGR Rwampara).

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For over two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements toward Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus disease (EVD) outbreak, which was then raging in North Kivu. The current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total estimated population of nearly 15 million, with massive internal displacement and cross-border flows to neighboring countries.

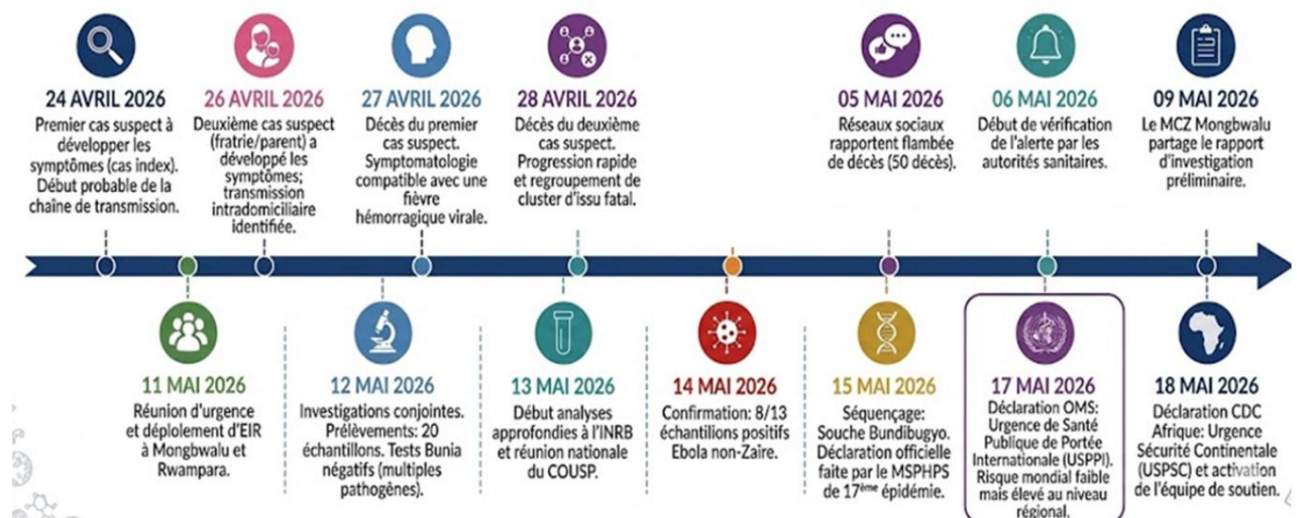


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 9, 2026

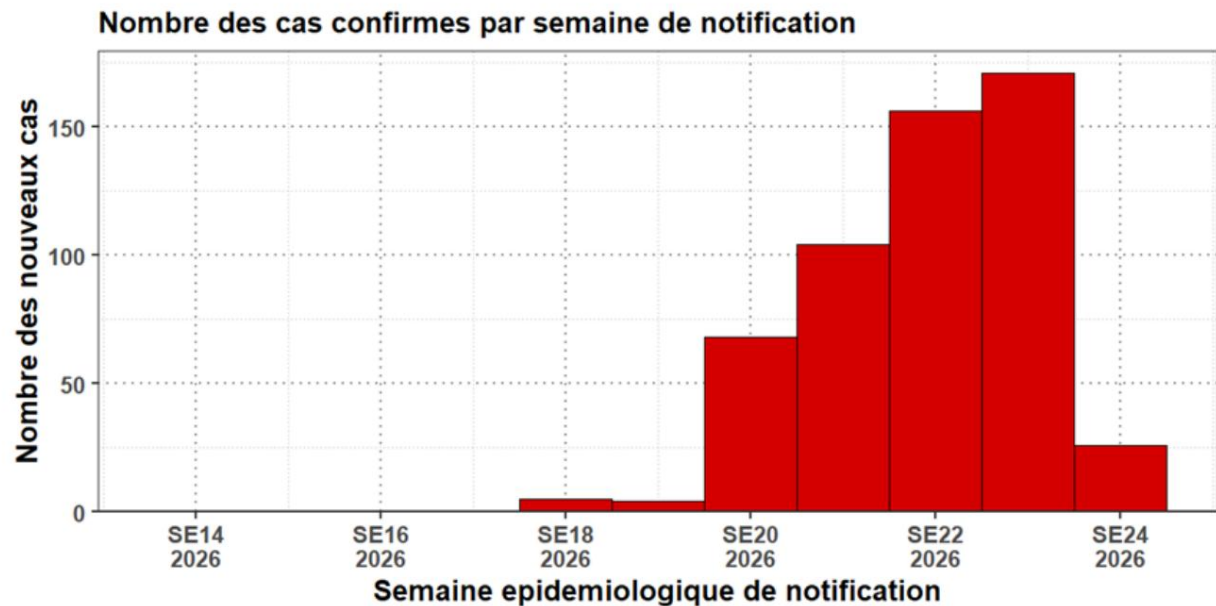
Province	Confirmed cases	Death (confirmed)	Lethality	Affected health zones	New case confirmed (24h)
Ituri	600	104	17.3%	18 out of 36 (50.0%)	37
North Kivu	32	22	68.8%	7 out of 34 (20.6%)	0
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Total	635	127	20.0%	26 out of 104 (25.0%)	37

- A new health zone has been affected, namely Tchomia (Ituri province). Thus, the number of affected health zones in Ituri has risen to 18 out of 36, and from 25 to 26 out of 104 for the country (section 3.1). As of June 9, 2026, Ituri province accounts for 94.4% of the confirmed cases reported in the three affected provinces. North Kivu province remains the most affected in terms of case fatality rate (68.8%), while Ituri province...
The situation remains stable in South Kivu province, which recorded its last confirmed case on May 26, 2026. The case fatality rate is low (17.3%) despite the large number of deaths recorded.

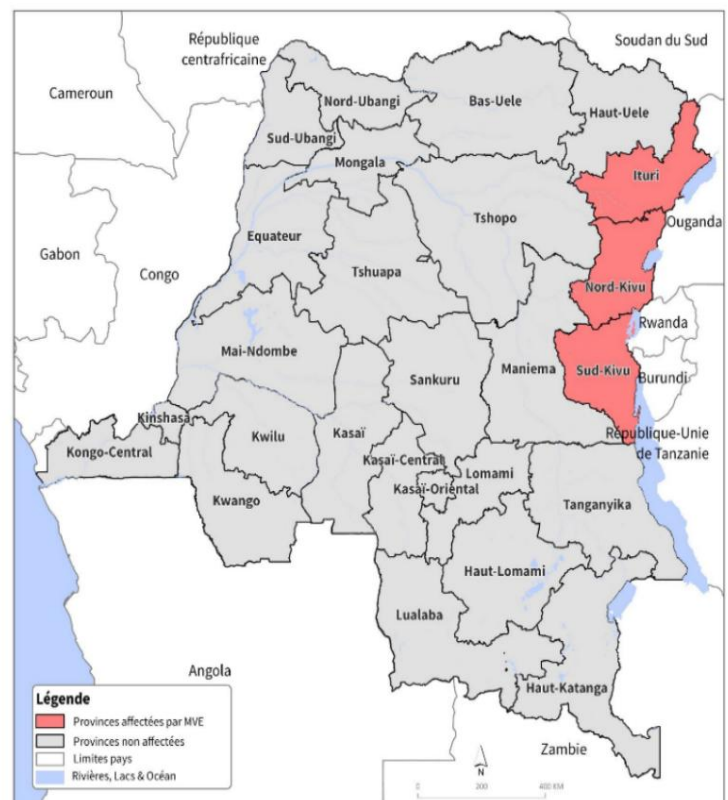
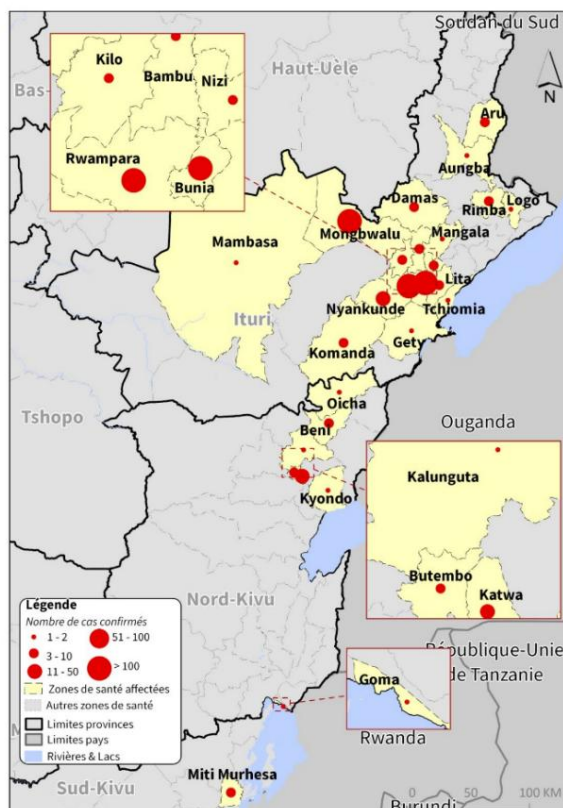
3.2 Distribution by health zone (Cumulative as of 09/06/2026)

- Ituri Province (600 cases):** Bunia (173), Rwampara (133), Mongbwalu (127), Nyankunde (32), Rimba (3), Mambasa (2), Bambu (5), Aru (3), Damas (3), Kilo (4), Nizi (5), Mangala (1), Aungba (2), Gety (1), Komanda (3), Lita (5), Logo (2) and Tchomia (2). Ninety-four (94) additional cases are being ventilated.
- North Kivu Province (32 cases):** Katwa (12), Beni (9), Butembo (6), Oicha (2), Kalunguta (1), Kyondo (1), Goma (1).
- South Kivu Province (3 cases):** Miti-Murhesa (3).

3.3 Time analysis: The number of confirmed cases is increasing week by week, indicating continued transmission of the disease within the community. A rapid geographical expansion of the epidemic is a real possibility if public health measures are not implemented quickly.



3.4 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zone and in the three affected provinces as of June 9, 2026

Table 1. Distribution of confirmed cases and deaths of Bundibugyo Ebola virus disease by health zone and province, Democratic Republic of Congo, as of June 9, 2026.

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Case fatality rate (%)
ITURI			
Bunia		15	8.7%
Rwampara	173,133	25	18.8%
Mongbwalu	127	47	37.0%
Nyankunde	32	1	3.1%
Bamboo	5	2	40.0%
Aru	3	1	33.3%
Kilo	4	1	25.0%
Nizi	5	0	0.0%
Mangala		0	0.0%
Damascus	1	0	0.0%
Aungba	3		50.0%
Gety	21	10	0.0%
Komanda	3	0	0.0%
Lita	5	0	0.0%
Logo	2	0	0.0%
Mambasa	2	1	50.0%
Rimba	3	0	0.0%
Tchomia	2	0	0.0%
Other ZS (undisclosed data)	94	10	10.6%
Ituri subtotal	600	104	17.3%
NORTH KIVU			
Katwa	12	8	66.7%
Blessed	9	7	77.8%
Butembo	6	4	66.7%
Oicha	2	2	100.0%
Kalunguta	1	1	100.0%
Kyondo	1	0	0.0%
Goma	1	0	0.0%
North Kivu subtotal	32	22	68.8%
SOUTH KIVU			
Miti-Murhesa	3	1	33.3%
South Kivu subtotal	3	1	33.3%
TOTAL	635	127	20.0%

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri:

- Organization of ceremonies marking the recovery and discharge of patients in the health zones of Nyankunde and Mongbwalu.
- On June 8th, a preparedness assessment mission was conducted in Kinshasa (Fiftieth Anniversary Hospital) by the Chief of Staff to His Excellency the Minister of Public Health, Hygiene and Social Welfare, in conjunction with the Public Health Emergency Operations Center.
- Continued support for the key elements of the response.
- Holding the daily coordination meeting with the PTFs and the members of the SGI.
- Finalization of CONOPS.

4.2 Epidemiological surveillance

Table 2. Management of epidemiological alerts (24h) as of June 9, 2026

Indicator		Ituri	North Kivu	South Kivu	Total
Alerts raised		78	526	12	616
Alerts investigated		57	505	12	574
Investigation rate		73.1%	96.0%	100.0%	93.2%
Alerts confirmed	Alive	38	50	1	89
	Deceased	11	19	0	30
Total suspected cases for today		49	69	1	119

As of June 9, 2026, **616 alerts** had been reported across the three affected provinces, **574 (93.2%)**

These alerts were investigated. Thus, **119 suspected cases (30 deaths) were recorded** in the 3 affected provinces.

Table 3. Contact tracing of confirmed cases as of June 9, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri	4,955	2,860	57.7%
North Kivu	841	597	71.0%
South Kivu	226	222	98.2%
Total	6022	3,679	61.1%

- Continuation of field surveillance activities (reporting alerts, investigations, active search, listing and monitoring contacts, retrospective documentary review of cases and deaths, etc.).
- Training of MCZs from the PEV Bunia branch on the operation of surveillance in the approach zonal,
- Joint mission for surveillance and care to assess isolation capacity.
- Deployment of senior epidemiologists in the Bunia and Rwampara health zones.

- 439 new contacts were listed, including 215 in Rwampara, 154 in Bunia and 70 in the Mongbwalu health zone while 406 contacts successfully completed their 21-day follow-up (381 in Nyankunde and 25 in Rwampara).

Update on checkpoint and point of entry (PoC/PoE) activities

- For June 9, 2026, 3 alerts were notified and validated, including 2 (dead bodies) at the PoC Dele/ZS Rwampara, and 1 alert (lifeless body) at PoC Mudzibala/ZS Bunia.

Table 4. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC) as of June 9, 2026

Indicators	Results
Proportion of activated PoCs	27.2% (15/55)
Number of travelers who passed through, % of travelers	24,324 98.2%
screened, % of travelers who washed their hands, % of travelers made aware of the risks	98.3%
Number of alerts notified	97.1%
Number of validated alerts	3
Number of validated alerts	3
% of results available in 24 hours	0%
Number of alerts transferred to the CT/CTE or for EDS	0
Number of contacts intercepted today	0

4.3 Laboratory

- **Ituri:** 54 new samples collected and analyzed (positivity 68.5%; n=37)
- **North Kivu:** 48 new samples collected, 12 of which were analyzed; all came back negative for Ebola virus disease as of June 9, 2026. 182 results remain pending due to insufficient reagents.
- **South Kivu:** 2 samples received and analyzed in the laboratory, all negative for Ebola virus disease.

4.4 Infection Prevention and Control (IPC)

- **Ituri:** IPC assessment of 9 healthcare facilities, including 5 in the Bunia health zone (score: 21% to 51.6%) and 4 in the Aru health zone (score: 16.6% to 33.3%); provision of handwashing kits to 154 TENASOSP and EXETAT examination centers; continued training of 50 healthcare providers (20 supervisors and 30 focal points) from 16 ESS of the Bunia ZS on the PCI; opening of the ring at CS Bora Uzima/ZS of Bunia (households, churches and ESS identified); briefing of 51 healthcare providers from 6 ESS in the ZS of Bunia on waste management, hand hygiene, bio-cleaning, wearing and removing PPE; briefing of 25 IT, 12 PPL and 3 TS from the ZS of Aru on waste management, bio-cleaning, wearing and removing PPE; decontamination of the Rwampara General Referral Hospital where suspected and confirmed cases of Ebola were admitted; briefing of 6 agents from the local red cross in the ZS of Bambu on EDS.
- **North Kivu:** Scorecard evaluation of the Katiri Health Center (69.4%) in the Kyondo Health Zone; the score obtained is 69.4%; an initial meeting was held with all partners involved in infection prevention and control (IPC) to map and harmonize activities in the Katwa Health Zone; decontamination of two health establishments (HEs) around the latest confirmed cases in the Butembo Health Zone; risk assessment of exposure for eight healthcare providers (6/8 high risk, all quarantined) in the Butembo Health Zone; evaluation

PCI of the MONUSCO site in Mubambiro and support for the team in setting up PCI measures and guidelines on the isolation of suspected cases; briefing of 6 service providers from CS Kasisi on the wearing and removal of PPE, risk assessment, cleaning and disinfection of premises.

4.5 Holistic care

- 8 new recoveries, including 7 in Nyankunde and 1 in Mongbwalu (Ituri province). Thus, the total number of recoveries has risen from 22 to 30, with 28 in Ituri province and 2 in North Kivu (Goma and Katwa).

Table 5. Patient movement within healthcare facilities (CTE/CT/CI) as of June 9, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	245	17	6	268
Total admissions (24h)	31	0	0	31
Exits (24h)				
Deceased (Suspects/Confessions)	7	0	0	7
Non-cases /	22	0	0	22
Recovered / Escaped (Suspects/Conf.)	9	0	0	9
Transferred to the HGR	1			
Total outflows	39	0	0	39
Patients in isolation (End of Day)	237	17	6	260
including confirmed (NC+AC)	111	5	1	117
and suspected cases	126	12		143
Overall occupancy rate	75.2% (237/315)	23.9% (17/71)	5 42.8% (6/14)	65.0% (260/400)

Mental health activities and psychosocial support

Table 6. Key indicators of mental health and psychosocial support activities as of June 9, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	12 (60.0%)	0%	0%
Confirmed cases currently being monitored that have benefited from SMSPS interventions	34 (37.3%)	3 (75.0%)	1 (100%)
New suspected cases that benefited from SMSPS interventions	12 (28.7%)	7 (58.3%)	0
Suspected cases under follow-up that have benefited from SMSPS interventions	16 (16.8%)	16 (72.7%)	5 (100%)
Laboratory results have been announced	24	1 (50.0%)	ND
Including positive ones	14 (56%)	1 (100.0%)	ND
Number of children separated in daycare who benefited from SMSPS interventions	1 (100%)	3 (100.0%)	3
Number of children separated in the community who benefited from SMSPS interventions	5	ND	3

Number of accompanying persons at the CTE who benefited from SMSPS interventions	37	ND	6
Number of PPLs who benefited from SMSPS interventions	6	8	0
Number of household and community members who benefited from SMSPS interventions	115	17	10
Patients who received dignity kits at the CTE	0	6 (85.7%)	1 (100%)

4.6 Continuity of services.

- Completion of the HGR Kilo evaluation.
- Development of a human resources recovery plan (frontline service providers).

4.7 CREC

- **North Kivu:** 11,303 people (6,401 women and 4,902 men) reached through mass awareness activities carried out in the community and 5,080 households visited, reaching 24,671 people sensitized; 649 community health workers mobilized, with coverage of 12 out of 20 health areas in the Karisimbi health zone; 340 people briefed on Ebola prevention in the Nyiragongo and Karisimbi health zones; 212 students, 6 teachers and supervisors sensitized in schools; 25 community alerts reported and 3 rumors collected and then clarified; 18 local radio stations engaged with 3 radio formats produced and broadcast; a rapid CREC assessment in the demobilized camp in Mubambiro in preparation for and response to Ebola with MONUSCO; descent to the PoC Mukulya/ZS of Beni to take stock of the situation in order to strengthen CREC actions in support of traveler screening.
- **South Kivu:** Organization of 2 mass awareness campaigns reaching 399 schoolchildren (168 girls) from Camp Panzi (ZS Ibanda) and Bwindi (Bagira) on prevention measures and signs of Ebola; support for the briefing of 45 community and religious relays in the Ibanda health zone with the support of the Methodist churches health coordination on Ebola and community-based surveillance; display of Ebola communication materials at ports and in strategic points of the city of Bukavu.
- **Ituri:** An alert was raised (a suspected case who had escaped from Mongbwalu and presented with hemorrhagic signs) by one of the traditional healers briefed by the CREC pillar. He facilitated the patient's transfer to the Ebola Treatment Center (AS Hoho, ZS Rwampara); a schedule was developed for broadcasting testimonies from those who recovered in Nyankunde on the local community radio station; messages on Ebola prevention measures were disseminated; 15 community dialogues were organized with 125 affected individuals, covering topics such as Ebola prevention measures, the dangers of self-medication, and the sequestration of suspected and/or confirmed cases; 139 people (52 men and 87 women) participated.
were reached during an educational talk; 9 organized mass awareness campaigns, 3,108 people affected (1,528 men and 1,580 women) in 7 health zones with themes on preventive measures for Ebola virus disease, the importance of EDS, the importance of community engagement, etc.; 4 advocacy meetings organized in the Rwampara health zone, 25 people took part (chairman of the IDP site, head of 10 Katanyibwa houses, military medical team from Kabanda).
- **Support for other pillars :** Surveillance: investigation and pre-listing of contacts (Mudzimaria Health Zone/Bunia Health Zone). IPC: Preparing families to participate in the DHS in the event of community deaths (Bunia Health Zone, Bankoko and Bigo Health Zones). **Result:** DHS completed.

Community feedback : (i) Complaints related to care at the CTE; (ii) Why are the EDS teams excessively slow in their interventions; (iii) What measures should be taken to

(iv) Avoid mass contamination at the central market where people are too crowded; (v) Make infection prevention and control (IPC) supplies (hand sanitizer, handwashing kits) available to the population; (v) The population fears that graduating students taking their state exams there could be a source of contamination. It would be better to find a center for them in Bunia (Boga Community Radio). These recommendations were formulated to provide appropriate and targeted responses.

4.8 Logistics and Security

- Delivery of PCI inputs, medicines and equipment to the health zones of Lolwa, Gety, Lita, Rethy, Mongbwalu, Mandima, Jiba, Drodoro, Komanda, Nyankunde, Fataki and Tchomia.
- 31 vehicles (2 functional ambulances) ensure the mobility of the coordination, field teams in the different ZS.

4.9. Security

- The security situation in the city of Bunia (Bunia and Rwampara health zones) is relatively calm. However, the activity of armed groups in the territories of Djugu, Irumu, and Mambasa continues to limit humanitarian access in several affected or at-risk health zones.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

- Briefing of 60 actors including WFP cooperating partners (38 participants: 14 women and 24 men) and service providers from the Kalangane and Nuru health centers in South Kivu: 22 women).
- Awareness-raising on the PEAHS for 20 (16 women and 4 men) faithful members of the St. Veronica Cell in the ZS of Bunia.

5. MAIN CHALLENGES

Challenge	Impact	Action required
No. 1 Reluctance to perform post-mortem sampling (swabs)	Confirmation delay; underestimation of cases	Intensive community engagement; involvement of religious leaders
2 Insufficient capacity in standardized CTES despite the opening of Rwampara	Potential saturation of existing CTES	Acceleration of the construction of planned CTES
3 Weakness in contact tracing (overall rate 61.1%, target 95%)	Interruption of compromised transmission chains	Training, deployment and motivation of community relays
4 Supply shortage Essential Generic Medicines (MEG) in targeted HGRs	Weakening of overall care	A plea for MEG funding
5 Insufficient PCI equipment (PPE, DLM, chlorine) in North Kivu	Nosocomial risk	Urgent supplies
6 Low increase in alerts in the 3 provinces	Delay in detection of case	Strengthening community surveillance
7 Insufficient financial resources for all pillars	Slowdown in operations	Resource mobilization (gap of USD 21.5 million)

6. PERSPECTIVES AND PRIORITY ACTIONS (24-72h)

Priority Action		Responsible pillar	Due date
	Training of service providers on the use of the RADIONE PCR	Laboratory	Day 2
1 2	Briefing of service providers on Ebola virus disease (EVD) and tuberculosis control (TBC) surveillance in affected and high-risk areas	Monitoring/CREC Day 3	
3	Briefing of community agents on Ebola surveillance and the SBC (Ituri province)	Monitoring/CREC Day 3	
4	Supervised support from the Director General of INSP in the technical pillars	Coordination	Continuous
5	Catch-up in contact tracing in Ituri remains weak.	Monitoring	Day 7
6	Strengthening infection prevention and control (IPC) measures and staff training in Nyankunde and other nosocomial areas	PCI	Day 2
7	Deployment of a standardized daily dashboard with KPIs per pillar	Coordination	Day 3
8	Plan for catching up on samples awaiting analysis in North Kivu Laboratory		Day 2
9	North Kivu: discharge of the recovered case in Goma (Day 1), response to security incidents (ADF incursion), IPC training at Virunga General Hospital, expansion of CREC to 15,000 households		Day 2
10	South Kivu: provision of 2 additional vehicles, installation of an internet kit in Miti-Murhesa, communication credits for the IT/ECZS		Day 2

Visit by the Chief of Staff of the Ministry of Public Health and Social Protection (MSPHPS) to assess preparedness for Ebola virus disease (EVD) at the Cinquanteaire Hospital in Kinshasa



**FOR ALL
INFORMATION**

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PLEASE CONTACT**

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